

METROPOLITAN GENERAL HOSPITAL

Department of Radiology & Diagnostic Imaging

Study Information

Patient:	John Michael Carter	MRN:	MGH-2024-087432
DOB:	14 March 1968 (Age: 57)	Accession #:	RAD-2026-00341
Study Date:	03 June 2026 10:30 AM	Report Date:	03 June 2026 12:05 PM
Ordering Physician:	Dr. Priya Nair, MD	Radiologist:	Dr. Sanjay Raman, MD, FRCR
Modality:	Chest X-Ray (PA and Lateral)	Priority:	STAT

Clinical Indication

57-year-old male with known heart failure (EF 35%) presenting with acute dyspnea, bilateral lower-extremity oedema, and SpO2 91% on room air. Evaluate for pulmonary oedema, pleural effusion, or pneumonia.

Technique

Portable PA and lateral chest radiographs were obtained in the radiology suite. Standard digital acquisition. No prior studies available for comparison at this institution.

Findings

Heart:	Cardiomegaly present. Cardiothoracic ratio estimated at 0.58 (upper limit of normal 0.50). No pericardial calcification identified.
Lungs:	Bilateral interstitial opacities with a perihilar 'bat-wing' distribution consistent with pulmonary oedema. Kerley B lines visible at both lung bases. No focal consolidation. No pneumothorax identified.
Pleural Spaces:	Bilateral pleural effusions present — moderate on the right, small on the left. Blunting of both costophrenic angles.
Mediastinum:	Superior mediastinum is widened. Aortic knuckle prominent — correlate with clinical history. Trachea midline.
Bones & Soft Tissues:	No acute osseous abnormality. Mild degenerative changes of the thoracic spine. Soft tissue unremarkable.
Devices:	No central venous catheter, pacemaker, or implantable device visible.

Impression

- Acute pulmonary oedema** — bilateral perihilar interstitial opacities with Kerley B lines, consistent with elevated pulmonary venous pressure in the context of known HFrEF.
- Cardiomegaly** — cardiothoracic ratio 0.58; consistent with known dilated cardiomyopathy.
- Bilateral pleural effusions** — moderate right, small left; likely transudative in the setting of decompensated heart failure.
- No acute pneumonia or pneumothorax** identified.

Recommendations

Clinical correlation and management as per treating cardiology team. Follow-up chest radiograph recommended after diuresis (48–72 hours) to assess treatment response. CT chest with contrast may be considered if clinical picture does not improve to exclude other pathology.

Reported by:	Dr. Sanjay Raman, MD, FRCR	Date:	03 Jun 2026 12:05 PM
Dictated:	03 Jun 2026 11:50 AM	Transcribed:	Automated voice recognition

This report has been electronically verified. Any queries should be directed to the duty radiologist at ext. 2210.